

TEST REPORT



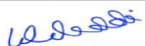
		Final Laboratory Report		PID : 7728856
Name : Mrs SASIKALA P	Sex/Age : Female / 41 Years	Lab ID : 60230800611		
Ref. By : SELF	SRF ID :	Ref. ID :		
Corporate :	UHID :			
Col Dt. Time : 04-Feb-2026 08:32	Recv Dt. Time : 04-Feb-2026 08:32	Sample Type : Whole Blood EDTA		
Reg Dt. Time : 04-Feb-2026 08:30	Report Released @ : 04-Feb-2026 11:23	Report Printed : 04-Feb-2026 13:37		

TEST	RESULTS	UNIT	BIOLOGICAL REF RANGE	REMARKS
Absolute Eosinophil Count	H 1400	Cells/ μ L	40 - 440	

Note:(LL-VeryLow,L-Low,H-High,HH-VeryHigh,A-Abnormal)

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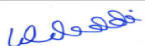
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Reg Dt. Time	: 04-Feb-2026 08:30	Report Released @	: 04-Feb-2026 11:24	Report Printed : 04-Feb-2026 13:37

TEST	RESULTS	UNIT	BIOLOGICAL REF RANGE	REMARKS
Peripheral Smear Examination				
RBC Morphology	No anisopoikilocytosis. The RBCs are normocytic normochromic.			
WBC Morphology	Increase in distribution with eosinophilia, monocytosis and toxic change in neutrophils.			
Platelet	Increase in distribution with a few megathrombocytes.			
Parasite	Not seen.			
Morphological Impression	1) EOSINOPHILIA WITH MONOCYTOSIS AND TOXIC CHANGE IN NEUTROPHILS. 2) REACTIVE THROMBOCYTOSIS. NO EVIDENCE OF HEMOPARASITES OR ABNORMAL WBCs ON THE SMEAR.			

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Reg Dt. Time : 04-Feb-2026 08:30	Report Released @ : 04-Feb-2026 11:23	Report Printed : 04-Feb-2026 13:37	

TEST	RESULTS	UNIT	BIOLOGICAL REF RANGE	TEST REMARK
Complete Blood Count				
Haemoglobin	12.6	gm/dL	12 - 16	
PCV(Calc)	38	%	37 - 42%	
RBC (Electrical Impedance)	4.6	millions/cmm	4.0 - 5.5	
Platelet Count	H 4,50,000	Cells/cmm	150000 - 400000	
Total WBC Count	H 13,000	Cells/cmm	4000 - 11000	
<u>Differential Count</u>				
Neutrophil	50	%	40 - 70	
Lymphocyte	L 29	%	30 - 40	
Eosinophil	H 09	%	2 - 4	
Monocytes	H 11	%	2 - 8	
Basophil	01	%	0 - 1	
ESR	H 60	mm/hour	5 - 20	

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Col Dt. Time : 04-Feb-2026 08:32	Recv Dt. Time : 04-Feb-2026 08:32	Sample Type : Urine F, Plasma Fluoride		
Reg Dt. Time : 04-Feb-2026 08:30	Report Released @ :	F,Whole Blood EDTA,Serum		

TEST	RESULTS	UNIT	BIOLOGICAL REF RANGE	REMARKS
Plasma Glucose - F <i>GOD/POD</i>	95	mg/dL	70 - 110	
Calcium <i>Arsenazo III</i>	9.1	mg/dL	8.4 - 10.2	
Vitamin D total - 25 hydroxy <i>CMIA</i>	21.9	ng/mL	Deficiency <20 Insufficiency 20-30 Sufficiency 30-100 Toxicity >100	

Glycated Haemoglobin Estimation

HbA1c <i>HPLC</i>	6.1	%	Below 6.0 Normal 6.1 - 7.0 Good Control 7.1 - 8.0 Fair Control 8.1 - 10.0 Unsatisfactory Above 10 Poor Control
Estimated Avg Glucose (3 Mths)	128.37	mg/dL	70 - 126 Diabetic >154

Interpretation :

HbA1C level reflects the mean glucose concentration over previous 8-12 weeks and provides better indication of long term glycemic control.
Levels of HbA1C may be low as result of shortened RBC life span in case of hemolytic anemia.
Increased HbA1C values may be found in patients with polycythemia or post splenectomy patients.
Patients with Homozygous forms of rare variant Hb(CC,SS,EE,SC) HbA1c can not be quantitated as there is no HbA.
In such circumstances glycemic control can be monitored using plasma glucose levels or serum Fructosamine.
The A1c target should be individualized based on numerous factors, such as age, life expectancy,comorbid conditions, duration of diabetes, risk of hypoglycemia or adverse consequences from hypoglycemia, patient motivation and adherence.

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Result Report

PatientID:

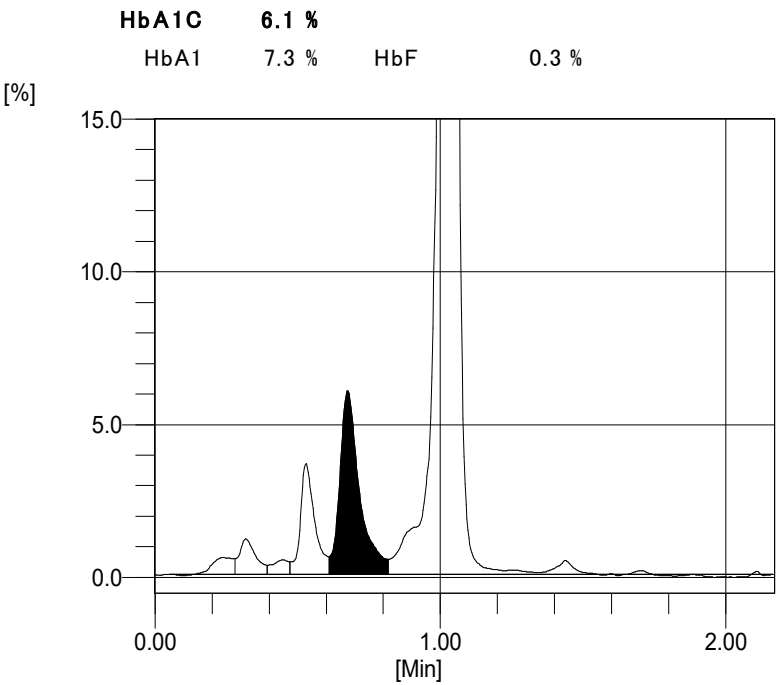
Record Date: 2026-02-04 12:37:03

Barcode: 260230800611

SampleNo: 02040049

CALIB : Y = 1.1575X + 0.6439

NAME	%	TIME	AREA
A1A	0.4	0.24	9.65
A1B	0.8	0.32	16.54
F	0.3	0.45	6.55
LA1C+	2.1	0.53	44.84
SA1C	6.1	0.68	101.63
A0	92.0	1.02	1997.17
TOTAL AREA			2176.38



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Corporate :				UHID :
Col Dt. Time : 04-Feb-2026 08:32	Recv Dt. Time : 04-Feb-2026 08:32	Sample Type : Serum		
Reg Dt. Time : 04-Feb-2026 08:30	Report Released @ : 04-Feb-2026 10:36	Report Printed : 04-Feb-2026 13:37		

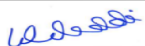
TEST	RESULTS	UNIT	BIOLOGICAL REF RANGE	REMARKS
Urea	19	mg/dL	15 - 40	
Uric Acid	3.4	mg/dL	2.6 - 6.2	
Creatinine <i>Enzymatic</i>	0.7	mg/dL	0.5-1.1	
Free T3 <i>CMIA</i>	2.99	pg/mL	1.71 - 3.71	
Free T4 <i>CMIA</i>	1.19	ng/dL	0.70 - 1.48	
TSH (ULTRASENSITIVE)				
TSH <i>CMIA</i>	3.60	μIU/mL	0.35 - 4.94	

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TEST	RESULTS	UNIT	BIOLOGICAL REF RANGE	REMARKS
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BIOCHEMICAL INVESTIGATIONS

Lipid Profile

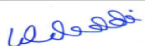
Cholesterol <i>CHOD-PAP</i>	190	mg/dL	Desirable: <200 Borderline: High: 200 - 239 High: > 240
Triglyceride	152	mg/dL	Normal: <150 Border Line High:150-199 High: 200 - 499 Very High: >500
HDL Cholesterol	40	mg/dL	<40 Low HDL >60 High HDL
LDL Cholesterol <i>Calculated</i>	119.60	mg/dL	Optimal <100 Near/Above Optimal 100-129 Borderline High 130-159 High 160-189 Very High >190
VLDL	30.40	mg/dL	< 30
Cholesterol/HDL-C	4.75	mg/dL	<3.5 - Low Risk 3.5 - 5.0 - Normal Risk >5.0 - High Risk

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Reg Dt. Time	: 04-Feb-2026 08:30	Report Released @	: 04-Feb-2026 12:11	Report Printed : 04-Feb-2026 13:37

TEST	RESULTS	UNIT	BIOLOGICAL REF RANGE	REMARKS
BIOCHEMICAL INVESTIGATIONS				
Bilirubin Total <i>DIAZO REACTION</i>	0.6	mg/dL	0.3 - 1.2	
Bilirubin-Direct	0.2	mg/dL	<0.3	
Bilirubin Indirect	0.40	mg/dL	0 - 0.8	
ALT (SGPT) <i>IFCC</i>	23	U/L	5-45	
AST (SGOT)	18	IU/L	15-37	
Alkaline Phosphatase <i>Amperometry</i>	66	U/L	New Born : 90 - 260 1 Month - 3 Yrs : 90 -180 3 Yrs - 10 Yrs : 130 - 260 10 Yrs - 14 Yrs : 130 - 340 > 14 Yrs : 30 - 130	
Gamma Glutamyl Transferase <i>G-glutamyl-carboxy-nitroanilide</i>	32	U/L	5-38	
Proteins (Total) <i>Biuret Method</i>	6.9	g/dL	6.4 - 8.2	
Albumin <i>(BCG)</i>	3.8	gm/dL	3.5 - 5.2	
Globulin	3.10	gm/dL	2.0-4.1	
A/G Ratio	1.23		1.0 - 2.1	

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Reg Dt. Time : 04-Feb-2026 08:30	Report Released @ : 04-Feb-2026 12:11	Report Printed : 04-Feb-2026 13:37		

TEST	RESULTS	UNIT	BIOLOGICAL REF RANGE	TEST REMARK
Colour	PALE YELLOW			
pH <i>Ion concentration</i>	6.5		4.6 - 8	
Reaction <i>Strip Method</i>	SLIGHTLY ACIDIC			
Appearance	CLEAR			
<u>Chemical Examination</u>				
Protein <i>Tetrabromophenol blue</i>	NIL		Negative	
Glucose Urine	NIL			
<u>Microscopic Examination - /HPF</u>				
Pus Cells	1 - 2	/HPF		
Red Blood Cell	1 - 2	/HPF	Nil	
Epithelial Cell	A FEW SQUAMOUS CELLS		-	
Cast	NIL		-	
Crystals	NIL		-	

----- End Of Report -----

For test performed on specimens received or collected from non-NDPL locations, it is presumed that the specimen belongs to the patient named or identified as labeled on the container/test request and such verification has been carried out at the point generation of the said specimen by the sender. NDPL will be responsible Only for the analytical part of test carried out. All other responsibility will be of referring Laboratory.

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